HALCYON RIDGE HEALTH SYSTEM | ENTERPRISE AI ENABLEMENT ADVISORY

Strategic Options & Advisory Analysis

Option B is narrowly preferred; the case rests on portfolio intent and specialist capacity, not on near-term financial return

Document	HRHS-AI-03	Version 2.1	November 2026 (supersedes v2.0)	Prepared by: Reginal Campbell, PMP
Audience and purpose	Executive Leadership Team and executive decision-makers. Compare three legitimate approaches and state a conditional recommendation.
Decision status	Provisional advisory recommendation (hypothetical). Conditional on capacity commitment (D-05), portfolio intent, and validations V-01 to V-07. No decision has been made.
Classification	FICTIONAL CASE STUDY. All organizational details, interviews, figures, dates, and findings are simulated scenario data, estimates, or labeled assumptions. No real engagement, approval, or result is claimed. Not a description of any real organization.

Summary of conclusions [J]

> **REVISED CONCLUSION (V2.0)**
>
> Option B scores 3.52 against 3.43 for Option A and 2.51 for Option C. That 0.09 gap is within judgment error; the conditions decide. On quantified three-year financials, Option A is about $1.4M cheaper than B, and no option repays its cost within three years in the low case. All options must meet the same mandatory privacy, security, and contractual obligations. Option B is provisionally recommended for what it adds beyond that floor: reusable controls, consistent risk evaluation, shared technical capabilities, and less duplication across future use cases. Those advantages are worth about $1.4M only if HRHS has a real pipeline beyond Wave 1 and specialist capacity is validated. If either fails, Option A is the better choice.

Decision objectives

The ELT asked for an approach that (1) produces measurable operating value in FY2027, (2) avoids unmanaged patient-data, safety, or compliance exposure, (3) fits constrained IT capacity and margin, and (4) avoids lock-in to an architecture HRHS must later unwind. These objectives conflict; the analysis makes the tradeoffs explicit.

The three options

Element	A. Targeted, Vendor-Led	B. Governed, Phased	C. Platform-First
Core idea	Buy AI embedded in qualified vendor products; govern each deployment.	Minimum shared governance and technical controls; prioritized use cases in gated waves.	Centralized AI platform, data foundations, and operating model before broad deployment.
First 12 months	Spend analytics module; P-02 denial add-on to production.	Governance; Wave 1 supply chain pilot; P-01/P-02 governed; AI gateway before any PHI use.	Platform, data pipelines, MLOps, center of excellence; first use case at month 9 to 12.
Architecture	Vendor-hosted; integration per product.	Shared gateway, logging, identity, evaluation; vendor or internal solutions plug in.	Enterprise platform with standardized hosting and pipelines.

Evaluation method and scoring

Each option is rated 1 (weak) to 5 (strong) on eleven criteria weighted by stated ELT priorities. Ratings are judgments [J] grounded in Document 02. **v2.0 change:** total cost re-rated A = 4 (was 3) for A's lowest three-year

cost. **v2.1 change:** security, privacy, compliance, and safety re-rated A = 3 (was 2). Mandatory controls apply under every option, so A meets the same compliance floor; the remaining difference is consistency of assurance and residual risk across more vendor surfaces, worth one point, not two. B's lead narrows from 0.21 to 0.09.

Criterion	Weight	A	B	C	Basis
Expected business value	15%	3	4	3	B lowers marginal cost of later use cases; value depends on a pipeline existing.
Time to initial value	10%	5	3	1	A: 4 to 6 months. B: pilot results ~Day 90. C: 9 to 12 months.
Total cost and investment	10%	4	3	2	3-year cost: A $6.35M; B $7.75M; C $12.3M.
Readiness fit	10%	4	3	1	C needs capabilities rated 1 to 2 (F-03, F-05, F-08).
Implementation complexity	8%	4	3	2	C builds platform, data, and operating model at once.
Security, privacy, compliance, safety	12%	3	4	4	All options meet mandatory controls; A applies them product by product, relies more on vendor evidence, and has more vendor surfaces (F-04).
Workforce and change impact	5%	3	4	3	B paces adoption with training per wave.
Vendor dependency and flexibility	8%	2	4	4	A concentrates dependency on vendor roadmaps.
Delivery capacity and execution risk	10%	4	3	1	B needs 4,600 specialist hours; C needs ~30,000 total hours.
Scalability and sustainability	7%	2	4	5	A creates little reusable capability.
Reversibility and exit	5%	3	4	2	B commits in tranches; C front-loads irreversible spend.
Weighted total (of 5.00)	100%	**3.43**	**3.52**	**2.51**	Differences below ~0.3 should be read as close.

Sensitivity and downside scenarios

Scenario	A	B	C
Base weights	3.43	3.52	2.51
Equal weights on all criteria	3.36	3.55	2.55
Speed-first: 8 points moved from safety to time to value	3.59	3.44	2.27
Single-wave: no use cases beyond Wave 1 (B value and scalability fall to 3)	3.43	3.30	2.51

[J] A overtakes B once about 3 weight points move from safety to speed, and A is preferred outright if HRHS has no credible pipeline beyond Wave 1. Both are leadership choices, not analytical errors, so the recommendation is explicitly conditional on them. No plausible reweighting makes C preferred; C depends on changed conditions.

Costs and capacity [E]

Estimates are order-of-magnitude (plus or minus 30%). Year-1 recurring cost is taken at 50% of steady state for all options to reflect ramp-up. Contracted capacity is included once, in one-time cost, at ~$110 per hour [A-04].

Element	A	B	C
One-time cost (incl. contracted capacity)	$1.6M	$3.0M	$6.8M
Steady-state recurring per year	$1.9M	$1.5M	$2.2M
FY2027 cash (one-time + 50% recurring)	$2.55M	$3.75M	$7.9M
3-year cost	$6.35M	$7.75M (incl. Wave 2)	$12.3M
IT hours, year 1 / uncommitted available	9,000 / 6,800	14,000 / 6,800	30,000 / 6,800

Element	A	B	C
Specialist hours needing release from ERP/EHR	~1,200	4,600	~11,000
Fits $4.5M envelope?	Yes	Yes ($0.75M contingency)	No

Option B build-up. One-time: governance and program office $0.45M; shared foundations (gateway, logging, identity integration, evaluation tooling) $0.90M; Wave 1 implementation and data remediation $0.85M; 7,200 contracted hours $0.80M. Recurring: licensing and consumption $0.70M; four support FTEs $0.66M; monitoring $0.15M. Wave 2 (UC-04) adds about $0.5M one-time and $0.25M per year and needs separate authorization. Option B 3-year cost: $3.75M (Y1) + $2.25M (Y2, incl. Wave 2 one-time) + $1.75M (Y3) = $7.75M.

Benefits: what is gross, net, recoverable, and when [E]

Benefit	Derivation	Type	Annual steady state
Contract utilization (UC-02)	Off-contract spend ~$186M (29% of $640M) [A-02]. Shift 3% to 5% to contract at 8% to 12% price differential. Shared attribution with sourcing actions.	Cost savings (P&L); realized as purchases occur	$0.45M to $1.1M gross
Preventable denials (UC-04)	Preventable pool 30% to 50% of $38M = $11.4M to $19.0M [A-03]. Reduce by 10% to 20% [A-07] = $1.1M to $3.8M gross, less ~10% rework and appeal labor.	Revenue recovery; cash lags 2 to 4 months	$1.0M to $3.4M net
Governance and risk reduction	Avoided privacy incidents and duplicate vendor spend.	Risk reduction; not monetized	Not quantified

v2.0 correction: v1.0 applied 5% to 10% to total write-offs ($38M) rather than the preventable portion, and did not net out rework cost. The corrected range is similar in size but rests on two stacked assumptions (A-03, A-07) and is low-confidence until V-03 is resolved.

Three-year view, including the downside case [E]

Case (FY2027 to FY2029)	3-yr cost	3-yr benefit	Net / Year-3 run-rate vs recurring
A: vendor-led (denial add-on live ~month 6)	$6.35M	$2.85M to $8.83M	Net -$3.50M to +$2.48M
B base: Wave 2 starts Year 2	$7.75M	$2.30M to $6.96M	Net -$5.45M to -$0.79M; run-rate -$0.30M to +$2.75M per year
B accelerated: P-02 passes at G4, productionized under controls	$7.75M	$2.85M to $8.83M	Net -$4.90M to +$1.08M
B downside: Wave 2 never authorized; recurring cut to $0.9M	$5.55M	$0.90M to $2.20M	Net -$4.65M to -$3.35M; run-rate -$0.45M to +$0.20M

Ramp assumptions: UC-02 at 25%/75%/100% of steady state in Years 1 to 3; denials at 20%/75%/100% when live by month 6, 0%/40%/100% when Wave 2 starts in Year 2.

[J] Three conclusions follow. First, quantified benefits do not justify Option B within three years in the low case; the decision is partly a judgment about the value of reusable controls and future pipeline. Second, **if Wave 2 is never authorized, the shared foundations are overbuilt** and Option B is financially the weakest choice; that is the single most important downside. Third, accelerating P-02 under controls closes most of B's timing gap with A, which is why the Day 90 plan evaluates P-02 for accelerated production rather than deferring it.

Risk and uncertainty

ID	Risk	Rating	Most exposed / mitigation
R-01	Unresolved PHI vendor risk: data retained or reused beyond permitted purposes (P-01 terms unverified; no violation identified)	High	All options; more vendor surfaces under A. MC-03 and MC-09 before any PHI use
R-02	Unmanaged AI adoption continues outside governance	High	A; MC-01 intake and procurement gate
R-03	Documentation errors in P-01 reach the medical record	Medium	All; MC-07 safety review and note audits
R-04	Output errors undetected after deployment	Medium	A; MC-05 and MC-08
R-05	Specialist capacity not released; plan not executable	High	B and C; D-05 and Document 08 recovery options
R-06	Architectural sprawl raises later consolidation cost	Medium	A; reference pattern via gateway
R-07	Benefits cannot be demonstrated or are smaller than case	High	All; baselines by Day 60, Finance validation
R-08	Workforce distrust undermines adoption	Medium	All; engagement, training, HR review
R-09	Vendor roadmaps make investments redundant	Medium	B and C; vendor briefings (V-06)
R-10	Governance becomes a bottleneck	Medium	B and C; tiered review, 5 to 10 day service levels

Recommendation [J]

CONDITIONAL RECOMMENDATION

Provisionally adopt Option B, authorizing only a 90-day mobilization and Tranche 1 ($1.5M). Proceed with Wave 1 only if named specialists are provisionally allocated by Day 15 and validated as available in the delivery window by Day 20 (D-05); otherwise apply Document 08. Wave 1: supply chain intelligence (UC-01, UC-02). Evaluate P-02 for accelerated production under controls at Day 90. Hold P-01 at current scope under clinical safety review. Return to the ELT at Day 90 with measured evidence and a restated financial case.

When the recommendation changes

Prefer	Conditions
Option A	Leadership does not intend more than one or two AI use cases in three years (V-07); or speed outweighs control in its risk posture; or specialist capacity cannot be validated and Document 08 recovery options fail; or qualified vendors accept HRHS data-use terms, narrowing the assurance gap further.
Option C	HRHS commits to more than 15 use cases in three years; ERP or EHR work completes early, freeing ~20,000 hours; incremental funding of ~$3.4M above envelope is available; leadership accepts value deferral into 2028.
Reduced B	Specialist capacity is partly available: sequence Wave 1 (UC-01 first) per Document 08.
Pause	P-01 vendor refuses amended terms and Privacy identifies a reportable issue, or a major security event diverts IT capacity.

Validation required before final commitment

ID	Question	Owner and timing
V-01	Is the $4.5M envelope available, and how much may be recurring?	CFO, before Dec 9
V-02	Is the 71% on-contract baseline accurate after item-master correction?	VP Supply Chain, Day 30

ID	Question	Owner and timing
V-03	What share of denial write-offs is preventable?	VP Revenue Cycle, Day 60
V-04	Will the P-01 vendor accept amended data-use terms?	CPO and General Counsel, Jan 31, 2027
V-05	Can named specialists be released, and when?	CIO/CTO with ERP and EHR sponsors, before Dec 9
V-06	Does the EHR vendor roadmap duplicate UC-07 or UC-08?	CMIO, Day 45
V-07	Does leadership intend to pursue use cases beyond Wave 1 within three years?	CEO and ELT, Dec 9